FOR HEALTHCARE PROFESSIONALS

Your patients' pain diary

One minute a day for the patient, two minutes of reading for you.

OUCH v0.11.2
Leaflet
9 November 2026

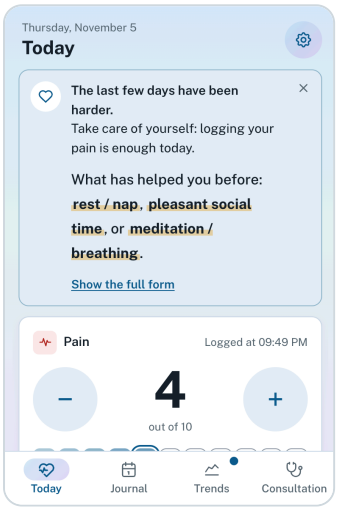
OUCH AT A GLANCE

A free, open-source pain diary for fibromyalgia and other chronic pain. The patient logs their day in under a minute on their phone, with no account; their data stays on the device. From it they produce a report for the appointment, which describes the data without interpreting it.

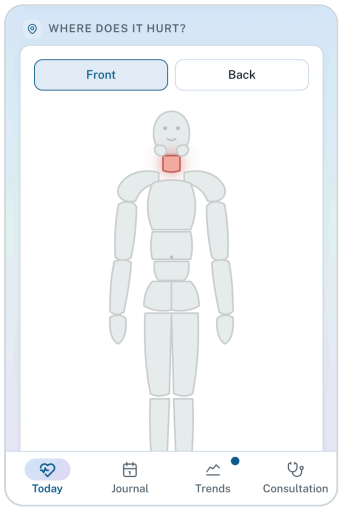
WHAT IT CHANGES IN THE CONSULTATION

Figures recorded day by day, rather than an account from memory. A comparison with the previous period, the days clearly more painful than usual, the treatments taken with the relief and side effects reported, and the question the patient wants to ask. Support for history-taking, not decision support.

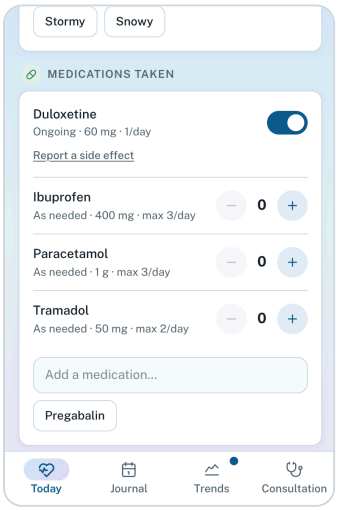
WHAT THE PATIENT ENTERS, IN UNDER A MINUTE



Hard days
Pain alone, in eleven boxes to tap.



Where, and how
Areas on the body outline, left or right side.



Treatments
Doses, as-needed doses, side effects.



The curve
7-day average, harder days shaded.

THE PATIENT LOGS

Pain of the day, 0 to 10: the only entry a day needs to count.
Numeric scale recommended for intensity[1][3]. Rated day by day: recalling a single day is more faithful than recalling the week[6].

Where it hurts, across 25 areas, left and right.
Location and spread, day by day. Self-reported, not a classification criterion.

Fatigue, sleep, brain fog, mood, stress, activity, 0 to 10.
Tracked alongside pain. Simple, unvalidated scales. Sleep and pain affect each other in both directions[14].

Depending on the condition: morning stiffness, night waking, interference, periods, headache...
Fourteen optional, unvalidated indicators, quickly reviewed in October 2026 (physiotherapist, pain psychologist, GP).

Treatments, regular or as needed.
Dose actually taken, missed days, as-needed doses, relief, dated side effects, dose changes.

What helped, weather, cycle, note, in one tap or optional.
The non-drug approaches the patient notices for themselves, and leads to test; the link between weather and pain remains inconsistent[7][11–13].

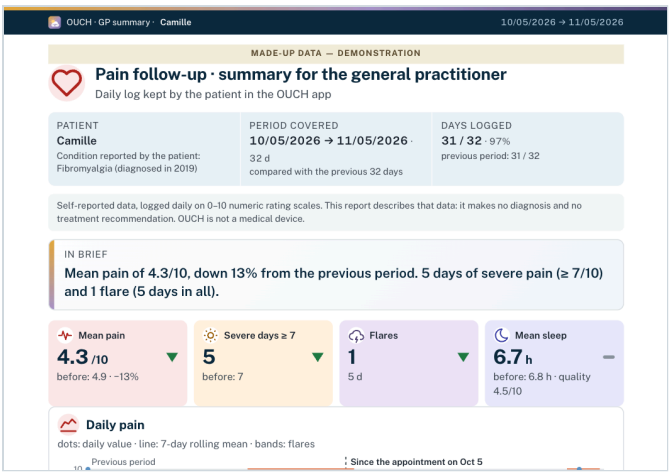
Hard days, a single entry

Pain alone is enough for a day to count. During a flare, the screen shrinks to pain; the rest is offered afterwards, without pressing.

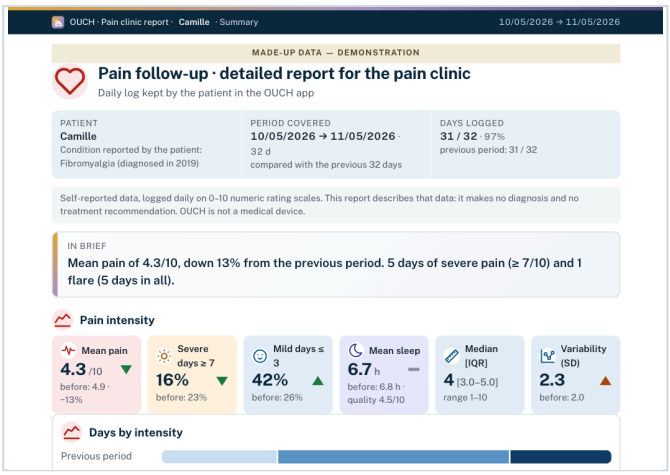
References numbered as in the methodology note: [1] IMMPACT · [3] HAS · [6] Broderick · [7][11–13] weather · [14] Finan.

Demonstration screenshots, fictitious data. OUCH is neither a medical device nor medical advice; the data is self-reported by the patient and unverified.

WHAT YOU RECEIVE



General practitioner: “In brief” sentence, key figures, curve



Pain clinic: the page 1 summary, before the six numbered parts

The patient chooses a version (family doctor or pain clinic). The report covers the time since their last appointment of that kind and compares it with the period before.

- **General practitioner**, 3 to 5 pages: “In brief” sentence, key figures with a trend arrow, curve, days by intensity compared with the earlier period, what happened, treatments, what the patient wants to raise, key points.
- **What the patient noticed**, in both: the leads from their diary they kept and their one-week trials, with no cause established.
- **What the patient added**: questions, note (optional), harder periods they noted, two periods compared.
- **Pain clinic (CETD)**, 6 to 10 pages: a summary, then six numbered parts (pain, treatments, symptoms and sleep, habits and context, patient questions and notes, method), with detailed statistics, harder days (flares) and the 3 days before each, with no causal claim, each symptom, painkillers week by week, associated factors.
- **In both**: the declared conditions (diagnosed or suspected), pain by day of the week with its chance band, the before and after of a treatment change, without concluding; the method.

Days without an entry are never filled in. The PDF is built on the phone; the name is not stored, the date of birth stays on the device. They can also hand over a one-page sheet (illnesses, treatments, allergies), declared by them.

WHAT OUCH DOES

- ✓ Describe: means, medians, distribution, trend.
- ✓ Spot the days clearly above the patient’s usual level, called “flares”.
- ✓ Cite the source of each threshold, and say when it is extrapolated.

WHAT OUCH DOES NOT DO

- × Make a diagnosis or advise a treatment.
- × Replace a validated questionnaire or your examination.
- × Present an association as a cause.

TRUST AND LIMITS

On the patient’s side

Data in their browser: no account, nothing sent apart from the optional weather lookup. Encrypted backup (password of their choice), optional code lock.

Open and tested

Free software (CeCILL v2.1), public code. 584 automated tests check the calculations and the app.

Frank about its limits

Self-reported data. Fatigue, sleep and mood scales unvalidated. The flare definition is OUCH’s own, with no consensus.

AT THE APPOINTMENT, IN THREE STEPS

1. The patient keeps their diary: one minute a day, pain alone on hard days.
2. Before the appointment, they generate the report: version, date of the last appointment of that kind, topics to raise.
3. You read it in two minutes. The methodology note details each formula, threshold and source.

Demonstration screenshots, fictitious data. OUCH is neither a medical device nor medical advice; the data is self-reported by the patient and unverified.

OUCH · maxauvy.github.io/OUCH
Free software, CeCILL v2.1 licence